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Name: إيرني واصف ابراهيم واصف National ID: 28708152302281	
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تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of uterine cancer for Whole-body PET/CT scan
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ابرينى

واصف ابراهيم واصف

مجاورة ١٨ - عمارة ٢ - مجموعة ٧
خمسة عشر مايو - القاهرة

١٩٨٧/٠٨



٢ ٨٧ ٠ ٨ ١٥ ٢٣ ٠ ٢٢ ٨١

LQ7766089

٢٠٢٦/٠٢ ٢٨٧٠٨١٥٢٣٠٢٢٨١

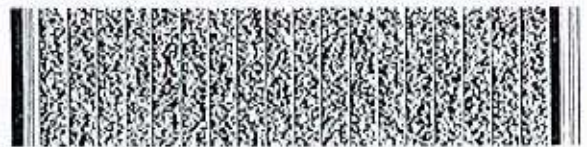
حاصلته على بكالوريوس خدمه اجتماعيه



انثى مسيحية متزوجة

ميشيل عريان عبدالمسيد اسكندر

البطاقة مارية حتى ٢٠٢٢/٠٢/٢٧



MDT FORM

Name:	Irene Wassef Ibrahim Wassef	MRN:	3695
Age:	39 Y	ID:	28708152302281
Menopausal Sta	POST	Phone:	01067358460
NRS SCORE:	0	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R

L

BX:

ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

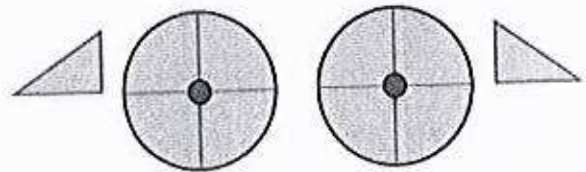
History:

STAGE 3 CANCER OVARY UNDERGONE TAH BSO 11/11/2025 HIGH GRADE SEROUS CARCINOMA BOTH OVARIES WERE INFILTRATED POSITIVE OMENTAL DEPOSITS PT IS ON ADJUVANT TAXOL CARBOPLATIN ,,BRCA 1 MUTANT FOR MDT CONCERNING NEED FOR OLAPARIB

Recommendations:

MDT Outcome & Plan

To add maintenance olaparib x 2 years (if no PD on platinum chemotherapy)
From NGO



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد التبركي				

Name: <u>ايريني واصف ابراهيم واصف</u>			
MR. : <u>3695</u>			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 38-year-old with stage 3 high-grade serous ovarian carcinoma who underwent a total abdominal hysterectomy and bilateral salpingo-oophorectomy on 11-11-2025. Post-operative chemotherapy was initiated with weekly Taxol 130 mg plus carboplatin AUC 2. The patient completed the first four cycles (W1–W4) before neutropenia prompted a one-week delay and consideration of prophylactic G-CSF. Subsequent cycles were administered as follows: W5–W6 with Neupogen 1 amp; W7 with Taxol 130 mg plus carboplatin AUC 2 and prophylactic Neupogen every two weeks; W8–W9 with the same regimen and ANC $3.65 \times 10^9/L$; W10–W12 with continued Taxol 130 mg plus carboplatin AUC 2; W13–W15 with the same regimen and monitoring of liver function, with sylimarine continued if hepatic enzymes remained elevated; W16–W18 with Taxol 130 mg plus carboplatin AUC 2, prophylactic Neupogen, and a plan to initiate olaparib for two years based on BRCA1 mutation status. The patient has also been receiving supportive care, including controloc and cetat at discharge, and has declined gastric prophylaxis or anti-emetics for liver support. Recent imaging on 27-06-2026 showed a few tiny pulmonary nodules and an L1 vertebral bony lesion; bone scan findings on 23-07-2026 indicated a small LV1 lesion likely a hemangioma rather than metastatic disease. The patient remains under consideration for ongoing olaparib therapy.			
تحريرا في ,			
التاريخ :		21-09-2026	
الوقت :		dina makram	
الطبيب المعالج :		يعتمد ,	

Patient Name : الأستاذ / إيريني واصف ابراهيم واصف	Request No. : 13003565
Patient No. : 28708152302281	Request Date : 23/11/2025
Path No. : SR-1685/25	Verification Date : 03/12/2025
Sex : Female	Financial Category : جوسف روسي - نفقة الدولة
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.
Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:
 Thirty referred blocks labelled (1008099). Together with, thirty-five referred (H&E) stained slides.

Microscopic Examination:

- * Procedure: Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration.
- Primary Tumor Site: Both ovaries.
- Laterality: Bilateral.
- Histologic Grade: High grade.
- Histologic Type: High-grade carcinoma, likely serous subtype.
- Description: Sheets, acini & papillae lined by highly pleomorphic malignant epithelial cells showing hyperchromatic nuclei.
- Mitosis: 20/10 Hpfs.
- Necrosis: Focal.
- Angio-invasion: Present.
- Ovarian Surface Involvement: Identified.
- Fallopian Tubes Involvement: Both tubes are involved from outside inward.
- Uterine body: Free.
- Cervix: Free.
- Vaginal cuff: Free.
- Appendix: Infiltrated by tumor.
- Omentum & peritoneum: Positive deposits of tumor nodules.
- Cytology: Positive for malignant cells.

Verified by :
 Dr.Duaa Mohamed



Patient Name : الأستاذ / إيريني واصف ابراهيم واصف	Request No. : 13003565
Patient No. : 28708152302281	Request Date : 23/11/2025
Path No. : SR-1685/25	Verification Date : 03/12/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Diagnosis:

Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):

- High grade carcinoma, likely serous carcinoma, involving (Both ovaries, both tubes, appendix, peritoneum & omentum).

- Free uterine body, cervix & vaginal cuff.

- Cytology: Positive for malignant cells.

* STAGE: pT3b.

* Immunophenotyping for (PAX-8, P53, ER,) is recommended.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 06/12/2025 09:32:10

Page 2 of 2

Printing By : All mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / ايريني واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة التولية
Age	: 38 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.

Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:

Thirty referred blocks labelled (1008099).

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; PAX8 clone (MRQ-50), P53 clone (Bp53-11) & ER clone (SP1).

Results:

- * IPT (Positive control showed appropriate reactivity):
- PAX-8: Shows diffuse strong positive nuclear staining in the tumor cells.
- P53: Strong diffuse positive staining, Mutant.
- ER: Weak patchy positive staining in tumor cells.

Diagnosis:

Right ovarian mass with massive pelvic peritoneal nodules, Referred blocks (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):

The immunohistochemical findings (Positive PAX-8, Mutant P53 & Weakly positive ER) are in keeping with high grade serous carcinoma of mullerian origin.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :



Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايريني واصف ابراهيم واصف

Age : 38 Y : 2 M : 2 D

Referred By Prof. Dr. : فؤاد عبد الشهيد

Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules

Nature of Specimen : Total abdominal hysterectomy & bilateral salpingo-oophorectomy,
Appendicectomy, Omentectomy with peritonectomy & Aspiration

Receiving Date : 2025-11-11

Analysis ID : 1008099

Gender : Female

Patient ID : 743010

PATHOLOGY REPORT

Gross :

Three designated containers were received as:

1) Total abdominal hysterectomy & bilateral salpingo-oophorectomy specimen,
Uterus measuring 6.5x6x3 cm. Endometrial thickness 0.3 cm, posterior outer surface of the
uterus showed multiple nodules, the largest measured 2x2x1.5 cm. Myometrial thickness 2 cm
and showed trabeculations.

Cervix 4 cm long, 3.5 cm in diameter with rubbery grayish white cut section.

Attached vaginal cuff was 1.5 cm.

RT ovary measuring 3.5x2.5x1 cm, ovarian mass measured 4x2x1.5 cm, cut section was
greyish white rubbery causing disruption of ovarian capsule, right tube measured 6 cm long,
showed fairly defined mass measured 12x7x4 cm, cut section was variegated greyish white
yellowish.

LT ovary measuring 3x2x1 cm, ovarian mass measured 2.5x2x1 cm, cut section was greyish
white rubbery causing disruption of the ovarian capsule, left tube measured 7 cm long.

Also received in the same container, Appendicectomy specimen measured 6 cm long, with 0.6
cm in diameter, maximal wall thickness was 0.3 cm, outer surface was glistening showing
multiple nodules measured up to 0.8x0.8 cm.

2) Omentectomy: Specimen measured 42x8 cm, showing multiple firm nodules measured up
to 1.8x1.8 cm.

Also received in the same container multiple peritoneal nodules measured up to 2x2 cm.

3) 16 cc of reddish fluid.

Microscopic :

Sections examined from the specimen (1) received revealed both ovaries showing infiltrating
tumour formed of malignant epithelial cells with moderate to marked nuclear pleomorphism
with scattered mitotic activity arranged in solid sheets, acini and papillae.

There is moderate desmoplasia.



عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٢٥٢١٧٠٠٥٢٨٧

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Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ابريني واصف ابراهيم واصف

Analysis ID : 1008099

Age : 38 Y : 2 M : 2 D

Gender : Female

Referred By Prof. Dr. : فؤاد عبد الشهيد

Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules

Nature of Specimen : Total abdominal hysterectomy & bilateral salpingeo-oophorectomy,
Appendicectomy, Omentectomy with peritonectomy & Aspiration

Receiving Date : 2025-11-11

Patient ID : 743010

PATHOLOGY REPORT

There is focal tumoural necrosis.

Ovarian capsule is disrupted.

Lympho-vascular space invasion is present.

Uterine wall revealed simple endometrial hyperplasia.

There is positive adherent peritoneal deposits.

Cervix is free.

Free vaginal cuff. The appendix is involved.

The tumor is adherent to outer surface of both tubes (right and left).

Sections examined from the specimen (2) received revealed omental tissue and peritoneal tissue positive for tumor deposits.

Cytological examination of the films and cell block prepared from the centrifugate of the fluid (3) received revealed eosinophilic proteinaceous material, red blood cells, lymphocytes, mesothelial cells and groups of cells with hyperchromatic nuclei.

Diagnosis :

Right ovarian mass with pelvic peritoneal nodules,

1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy & Appendicectomy. BILATERAL OVARIAN HIGH GRADE CARCINOMA LIKELY HIGH GRADE SEROUS CARCINOMA, INVOLVING BOTH TUBES FROM OUTSIDE, POSITIVE ADHERENT PERITONEUM, INVOLVED APPENDIX, FREE UTERINE WALL, FREE CERVIX, FREE VAGINAL CUFF.

2) Omentectomy with peritonectomy, POSITIVE FOR TUMOR DEPOSITS.

3) Aspiration, Cytology, POSITIVE FOR MALIGNANCY.

- TUMOR STAGE: pT3b.

- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



Shady Elia Anis

This report is electronically signed, any alteration or modification would render it void.

عمارة رمسيس - بنوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ١٢٨٧٠٠٥٢١٧

Name : Eriny Wassef Ibrahim Wassef

Date : 20-12-2025 11:14

Branch : Portable

Medical No. : GRI-133408

Ref.Physician Prof :

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **No available history?! Metastatic work up.**

Findings:

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

OPINION:

- **Free metastatic work up.**

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

Name : Eriny Wassef Ibrahim Wassef

Date : 20-12-2025 11:14

Branch : Portable

Medical No. : GRI-133408

Ref.Physician Prof :

Ins.Company :

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Uterus could not be seen, likely surgically removed.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No suspicious osseous lesions.

Opinion:

- Free metastatic work up.

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Dr. Mai Shaaban

Techniques:

Sagittal and Axial T1 & T2 W Sequences

MRI OF Lumbo-Sacral spine.REPORT

- Normal alignment of the lumbar spine, with no signs of instability detected.
 - *LVI small vertebral body lesion showed high T1 and high T2 likely small hemangioma rather than being suspicious for further bone scan assessment.*
 - Normal marrow signal of the other lumbar vertebrae, with no other focal lesions identified.
 - *LVS/SVI small left posterior annular fissure is noted.*
 - No significant disc bulges or herniations seen, with no thecal or foraminal compromise observed.
 - Normal site, shape and intensity of the conus medullaris and cauda equina roots.
 - No significant bony canal tightness seen.
 - Normal pre and paravertebral soft tissue structures.
- OPINION:**
- *LVI small vertebral body lesion showed high T1 and high T2 likely small hemangioma rather than being suspicious for further bone scan /PET CT assessment.*

Dr.Redu Tag

Patient's Name: Irene Wasef Ibrahim Wasef
Date: 2026-06-06
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Status post TAH-BSO for neoplastic lesion.

I- CHEST:

- Few tiny bilateral scattered pulmonary nodules largest measuring 5 mm at the right middle lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Status post TAH-BSO showing clear operative bed.
- Mildly enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Left internal iliac lymph node measuring 13 x 10 mm.
- No ascites.
- L1 vertebral body small lytic focus.

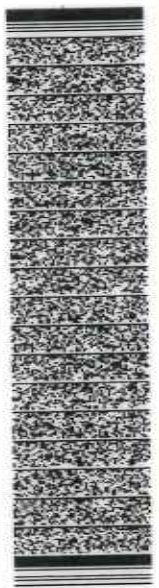
Opinion:

- ❖ Few tiny bilateral scattered pulmonary nodules for follow up.
- ❖ L1 vertebral body small lytic focus for bone scan.
- ❖ PET-CT is advised.

Dr.sherif Wagdy

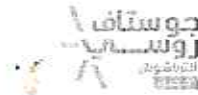


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حاصلته على بكالوريوس خدمة اجتماعية
أنثى مسيحية متزوجة
ميشيل عريان عبدالحميد السكندر
البطاقة سارية حتى ٢٠٢٣/٠٢/٢٧



Patient's Name: ERINYWASEF
Date: 2026-04-08
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

٣٧٩٥



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 38-year old female with negative family history and pathologically proven ovarian cancer , screening study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance.
- Multiple circumscribed oval parallel homogenous hypoechoic benign looking soldi mass
- No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parechyma with mottled appearance yet with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): breast adenosis.

Left breast (BIRADS 2): breast adenosis, for hormonal assay.

Best Regards,
Dr., Amal Hegazi, M.Sc.

اسم المريض: د. نورا
عنوان المريض: د. نورا
مدير الوحدة: د. نورا
طبيب الوحدة: د. نورا



رئيس مجلس الإدارة
د. نورا

مدير المعامل
د. نورا

PATIENT NAME أحمد محمد إبراهيم			Registered 26-10-2025 16:23:05	Collected 26-10-2025 16:31:08
			Authenticated 27-10-2025 00:48:46	Printed 27-10-2025 07:18:52
Visit Number 20725545005	Age 38 Year	Gender Female	Referred By Prof. Dr. Noura	Client ID 1618

Test Name	Result	Unit	Reference Range	Previous Result
Kidney Function Tests				
Serum Creatinine	0.72	mg/dL	0.5 - 1.1	
Serum Urea	22	mg/dL	13 - 43	
Tumor Markers				

Alpha-fetoprotein (AFP)	2.0	ng/mL	Up to 8	
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Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.

Carcinoembryonic Antigen (CEA)	0.67	ng/mL	Non smoker: up to 3.0 Smoker: up to 5.0	
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Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.

CA 125	740.00	U/mL	0 - 35	
--------	--------	------	--------	--

Follow up is recommended

Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.

Dr. ASD El-Aziz El-Nokaly
Professor of Medical Bio-Chemistry,
Faculty of Medicine, Al Azhar University

1-1

Patient Name	: الأستاذ / ايريني واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

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- * Procedure: Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration.
- Primary Tumor Site: Both ovaries.
- Laterality: Bilateral.
- Histologic Grade: High grade.
- Histologic Type: High-grade carcinoma, likely serous subtype.
- Description: Sheets, acini & papillae lined by highly pleomorphic malignant epithelial cells showing hyperchromatic nuclei.
- Mitosis: 20/10 Hpfs.
- Necrosis: Focal.
- Angio-invasion: Present.
- Ovarian Surface Involvement: Identified.
- Fallopian Tubes Involvement: Both tubes are involved from outside inward.
- Uterine body: Free.
- Cervix: Free.
- Vaginal cuff: Free.
- Appendix: Infiltrated by tumor.
- Omentum & peritoneum: Positive deposits of tumor nodules.
- Cytology: Positive for malignant cells.

Verified by :
Dr.Duaa Mohamed



Patient Name	: الأستاذ / ايريني واصف ابراهيم واصف	Request No.	: 13003565
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Age	: 38 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:

Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):

- High grade carcinoma, likely serous carcinoma, involving (Both ovaries, both tubes, appendix, peritoneum & omentum).

- Free uterine body, cervix & vaginal cuff.

- Cytology: Positive for malignant cells.

* STAGE: pT3b.

* Immunophenotyping for (PAX-8, P53, ER,) is recommended.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 06/12/2025 09:32:10

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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Dr.

Shady Elia Anis

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Patient Name : إيريني واصف ابراهيم واصف

Age : 38 Y : 2 M : 2 D

Referred By Prof. Dr. : فؤاد عبد الشهيد

Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules

Nature of Specimen : Total abdominal hysterectomy & bilateral salpingo-oophorectomy,
Appendectomy, Omentectomy with peritonectomy & Aspiration

Receiving Date : 2025-11-11

Analysis ID : 1008099

Gender : Female

Patient ID : 743010

PATHOLOGY REPORT

Gross :

Three designated containers were received as:

1) Total abdominal hysterectomy & bilateral salpingo-oophorectomy specimen,
Uterus measuring 6.5x6x3 cm. Endometrial thickness 0.3 cm, posterior outer surface of the
uterus showed multiple nodules, the largest measured 2x2x1.5 cm. Myometrial thickness 2 cm
and showed trabeculations.

Cervix 4 cm long, 3.5 cm in diameter with rubbery grayish white cut section.

Attached vaginal cuff was 1.5 cm.

RT ovary measuring 3.5x2.5x1 cm, ovarian mass measured 4x2x1.5 cm, cut section was
greyish white rubbery causing disruption of ovarian capsule, right tube measured 6 cm long,
showed fairly defined mass measured 12x7x4 cm, cut section was variegated greyish white
yellowish.

LT ovary measuring 3x2x1 cm, ovarian mass measured 2.5x2x1 cm, cut section was greyish
white rubbery causing disruption of the ovarian capsule, left tube measured 7 cm long.

Also received in the same container, Appendectomy specimen measured 6 cm long, with 0.6
cm in diameter, maximal wall thickness was 0.3 cm, outer surface was glistening showing
multiple nodules measured up to 0.8x0.8 cm.

2) Omentectomy: Specimen measured 42x8 cm, showing multiple firm nodules measured up
to 1.8x1.8 cm.

Also received in the same container multiple peritoneal nodules measured up to 2x2 cm.

3) 16 cc of reddish fluid.

Microscopic :

Sections examined from the specimen (1) received revealed both ovaries showing infiltrating
tumour formed of malignant epithelial cells with moderate to marked nuclear pleomorphism
with scattered mitotic activity arranged in solid sheets, acini and papillae.

There is moderate desmoplasia.



عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
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PATHOLOGY REPORT

There is focal tumoural necrosis.

Ovarian capsule is disrupted.

Lympho- vascular space invasion is present.

Uterine wall revealed simple endometrial hyperplasia.

There is positive adherent peritoneal deposits.

Cervix is free.

Free vaginal cuff. The appendix is involved.

The tumor is adherent to outer surface of both tubes (right and left).

Sections examined from the specimen (2) received revealed omental tissue and peritoneal tissue positive for tumor deposits.

Cytological examination of the films and cell block prepared from the centrifugate of the fluid (3) received revealed eosinophilic proteinaceous material, red blood cells, lymphocytes, mesothelial cells and groups of cells with hyperchromatic nuclei.

Diagnosis :

Right ovarian mass with pelvic peritoneal nodules,

1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy & Appendectomy. BILATERAL OVARIAN HIGH GRADE CARCINOMA LIKELY HIGH GRADE SEROUS CARCINOMA, INVOLVING BOTH TUBES FROM OUTSIDE, POSITIVE ADHERENT PERITONEUM, INVOLVED APPENDIX, FREE UTERINE WALL, FREE CERVIX, FREE VAGINAL CUFF.

2) Omentectomy with peritonectomy, POSITIVE FOR TUMOR DEPOSITS.

3) Aspiration, Cytology, POSITIVE FOR MALIGNANCY.

- TUMOR STAGE: pT3b.

- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



Shady Elia Anis

This report is electronically signed, any alteration or modification would render it void.

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تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Patient Name	: الأستاذ / إيريني واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.

Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:

Thirty referred blocks labelled (1008099).

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; PAX8 clone (MRQ-50), P53 clone (Bp53-11) & ER clone (SP1).

Results:

- * IPT (Positive control showed appropriate reactivity):
- PAX-8: Shows diffuse strong positive nuclear staining in the tumor cells.
- P53: Strong diffuse positive staining, Mutant.
- ER: Weak patchy positive staining in tumor cells.

Diagnosis:

Right ovarian mass with massive pelvic peritoneal nodules, Referred blocks (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendectomy, Omentectomy with peritonectomy & Aspiration):

The immunohistochemical findings (Positive PAX-8, Mutant P53 & Weakly positive ER) are in keeping with high grade serous carcinoma of mullerian origin.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :



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Referring Doc.		Registered by	,marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

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